HEALTH SYSTEMS, PUBLIC HEALTH AND UNIVERSAL HEALTH COVERAGE

COVERAGE CUBE → POLICY AND MISSION CARD → TWO PILLARS KEPT APART → TWO ENROLMENT ROUTES → SOURCE DISCIPLINE CARD → IMPOVERISHMENT MECHANISM

Read the thick cyan rail from Stage 00 to the final core synthesis. Each card uses a concept-appropriate structure; the grey E card is optional enrichment only and never repairs missing core content.

◆ PRIMARY CORE FLOW ◆ SUBORDINATE EXTRA ◆ APPROVAL / REVIEW

CORE BEFORE EXTRA • prior artifacts unchanged • exact same master drives poster and tiles

Approval: FALSE • Pending user review • active generation g1 • explicit approval of this exact generation is required • all prior artifacts unchanged

00

STAGE 00

COVERAGE CUBE

COVERAGE CUBE | WHO IS COVERED, ALL CITIZENS OR IDENTIFIED BENEFICIARIES | PROMOTIVE, PREVENTIVE, CURATIVE AND REHABILITATIVE CARE | FINANCIAL PROTECTION | HOW LITTLE THE HOUSEHOLD PAYS AT THE POINT OF | A SYSTEM CAN WIDEN ONE EDGE WHILE NARROWING ANOTHER

SOCIAL STRUCTURE / CONCEPT

- POPULATION → breadth: who is covered, all citizens or identified beneficiaries
- SERVICES → depth: promotive, preventive, curative and rehabilitative care

MECHANISM OF CHANGE

- FINANCIAL PROTECTION → height: how little the household pays at the point of care
- RULE → a system can widen one edge while narrowing another; test all three

CONSEQUENCE / LIMIT

- POPULATION → breadth: who is covered, all citizens or identified beneficiaries
- SERVICES → depth: promotive, preventive, curative and rehabilitative care

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → a system can widen one edge while narrowing another; test all three.

01

STAGE 01

POLICY AND MISSION CARD

POLICY AND MISSION CARD | POLICY 2017 | RAISE PUBLIC HEALTH EXPENDITURE TO 2.5 PER CENT OF | ITS STATUS | A POLICY DOCUMENT, NOT AN ENACTED STATUTE; COMMITMENTS, NOT | RURAL MISSION | LAUNCHED 12 APRIL 2005 FOR THE RURAL POPULATION AND | URBAN MISSION

POLICY 2017 → raise public health expenditure to 2.5 per cent of gross domestic product → **ITS STATUS → a policy document, not an enacted statute; commitments, not entitlements** → **RURAL MISSION → launched 12 April 2005 for the rural population and vulnerable groups** → **URBAN MISSION → Union Cabinet decision dated 1 May 2013 created it as a sub-mission** → **SPECIAL FOCUS → Empowered Action Group States, North East, Jammu and Kashmir, Himachal**

OFFICIAL EVIDENCE / DATE

- POLICY 2017 → raise public health expenditure to 2.5 per cent of gross domestic product
- ITS STATUS → a policy document, not an enacted statute; commitments, not entitlements

WHAT IT PROVES

- RURAL MISSION → launched 12 April 2005 for the rural population and vulnerable groups
- URBAN MISSION → Union Cabinet decision dated 1 May 2013 created it as a sub-mission

CLAIM LIMIT

- SPECIAL FOCUS → Empowered Action Group States, North East, Jammu and Kashmir, Himachal
- BENCHMARK → facilities measured against the Indian Public Health Standards

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: BENCHMARK → facilities measured against the Indian Public Health Standards.

02

STAGE 02

TWO PILLARS KEPT APART

TWO PILLARS KEPT APART | PILLAR ONE | WELLNESS CENTRES AND HEALTH MANDIRS; COMPREHENSIVE PRIMARY HEALTH CARE | ITS PACKAGE | TWELVE SERVICES INCLUDING NON-COMMUNICABLE DISEASE AND MENTAL HEALTH | ITS STAFF | COMMUNITY HEALTH OFFICER OR MID-LEVEL HEALTH PROVIDER | PILLAR TWO

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care	ITS STAFF → community health officer or mid-level health provider	ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals
ITS PACKAGE → twelve services including non-communicable disease and mental health	PILLAR TWO → cashless secondary and tertiary hospitalisation cover	PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care

SOCIAL UNIT / DEFINITION

- PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care
- ITS PACKAGE → twelve services including non-communicable disease and mental health

DECISIVE DISTINCTION

- ITS STAFF → community health officer or mid-level health provider
- PILLAR TWO → cashless secondary and tertiary hospitalisation cover

EXAM TRAP / LIMIT

- ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals
- PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals.

03

STAGE 03

TWO ENROLMENT ROUTES

TWO ENROLMENT ROUTES | CORE ROUTE | ELIGIBILITY-BASED, IDENTIFIED THROUGH CENSUS DEPRIVATION CRITERIA | AGE ROUTE | EVERY CITIZEN AGED SEVENTY AND ABOVE MAY ENROL REGARDLESS | THE AGE ROUTE IS UNIVERSAL ONLY FOR THAT AGE | ERROR TO AVOID | WRITING AN AGE-SPECIFIC EXPANSION UP AS UNIVERSAL COVER FOR

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
CORE ROUTE → eligibility-based, identified through census deprivation criteria	PRECISION → the age route is universal only for that age group	CORE ROUTE → eligibility-based, identified through census deprivation criteria
AGE ROUTE → every citizen aged seventy and above may enrol regardless of income	ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages	AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

SOCIAL UNIT / DEFINITION

- CORE ROUTE → eligibility-based, identified through census deprivation criteria
- AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

DECISIVE DISTINCTION

- PRECISION → the age route is universal only for that age group
- ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages

EXAM TRAP / LIMIT

- CORE ROUTE → eligibility-based, identified through census deprivation criteria
- AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages.

04

STAGE 04

SOURCE DISCIPLINE CARD

SOURCE DISCIPLINE CARD | HEALTH AUTHORITY | IMPLEMENTS THE HOSPITALISATION PILLAR; EMPANELS AND PAYS CLAIMS | HEALTH ACCOUNTS | OFFICIAL ESTIMATES OF PUBLIC, PRIVATE AND OUT-OF-POCKET SPENDING | 47.1 PER CENT IN 2019-20; 44.4 PER CENT IN | NEVER CONFUSE THE PUBLICATION DATE WITH THE ACCOUNTING YEAR | A HIGH SHARE SIGNALS WEAK PUBLIC PROVISION, NOT HIGH

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims	SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22	MEANING → a high share signals weak public provision, not high utilisation
HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending	RULE → never confuse the publication date with the accounting year	HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

SOCIAL UNIT / DEFINITION

- HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims
- HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending

DECISIVE DISTINCTION

- SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22
- RULE → never confuse the publication date with the accounting year

EXAM TRAP / LIMIT

- MEANING → a high share signals weak public provision, not high utilisation
- HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → never confuse the publication date with the accounting year.

05

STAGE 05

IMPOVERISHMENT MECHANISM

IMPOVERISHMENT MECHANISM | CARE IS SOUGHT FROM A PROVIDER OUTSIDE THE FUNDED | OUT-OF-POCKET PAYMENT AT THE POINT OF SERVICE | CATASTROPHIC SPENDING | ABOVE A THRESHOLD SHARE OF HOUSEHOLD INCOME OR CONSUMPTION | ASSET SALE OR DEBT | YEARS OF ACCUMULATION ARE UNDONE BY ONE HOSPITALISATION | THIS IS WHERE THE HEALTH OWNER MEETS THE POVERTY

ILLNESS → care is sought from a provider outside the funded public layer → **OUT-OF-POCKET PAYMENT at the point of service** → **CATASTROPHIC SPENDING: above a threshold share of household income or consumption** → **ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation** → **LINK → this is where the health owner meets the poverty owner**

SOCIAL STRUCTURE / CONCEPT

- ILLNESS → care is sought from a provider outside the funded public layer
- OUT-OF-POCKET PAYMENT at the point of service

MECHANISM OF CHANGE

- CATASTROPHIC SPENDING: above a threshold share of household income or consumption
- ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation

CONSEQUENCE / LIMIT

- LINK → this is where the health owner meets the poverty owner
- ILLNESS → care is sought from a provider outside the funded public layer

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: LINK → this is where the health owner meets the poverty owner.

06

STAGE 06

REFERRAL SPINE

REFERRAL SPINE | FIRST CONTACT; OUTREACH AND COMMUNITY-LEVEL SERVICES | PRIMARY HEALTH CENTRE | PREVENTIVE, PROMOTIVE AND BASIC CURATIVE CARE | COMMUNITY HEALTH CENTRE | SPECIALIST OUTPATIENT AND INPATIENT CARE | DISTRICT HOSPITAL | SECONDARY CARE AND THE GATEWAY TO TERTIARY REFERRAL

SUB-CENTRE → first contact; outreach and community-level services

PRIMARY HEALTH CENTRE → preventive, promotive and basic curative care → **COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care** → **DISTRICT HOSPITAL → secondary care and the gateway to tertiary referral**

SOCIAL STRUCTURE / CONCEPT

- SUB-CENTRE → first contact; outreach and community-level services
- PRIMARY HEALTH CENTRE → preventive, promotive and basic curative care

MECHANISM OF CHANGE

- COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care
- DISTRICT HOSPITAL → secondary care and the gateway to tertiary referral

CONSEQUENCE / LIMIT

- USE → name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ
- SUB-CENTRE → first contact; outreach and community-level services

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: USE → name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ.

07

STAGE 07

MARKETISATION AND ITS HARMS

MARKETISATION AND ITS HARMS | PRIVATE DOMINANCE IN CURATIVE CARE | OFTEN UNREGULATED OR UNDER-REGULATED | HIGH COSTS AND UNNECESSARY PROCEDURES | CATASTROPHIC SPENDING AND INEQUITABLE ACCESS | DO NOT DENY PRIVATE CAPACITY, AND DO NOT ASSUME

PRIVATE DOMINANCE IN CURATIVE CARE → often unregulated or under-regulated → **HIGH COSTS and unnecessary procedures** → **CATASTROPHIC SPENDING and inequitable access** → **BALANCE → do not deny private capacity, and do not assume regulation is automatic**

SOCIAL STRUCTURE / CONCEPT

- PRIVATE DOMINANCE IN CURATIVE CARE → often unregulated or under-regulated
- HIGH COSTS and unnecessary procedures

MECHANISM OF CHANGE

- CATASTROPHIC SPENDING and inequitable access
- BALANCE → do not deny private capacity, and do not assume regulation is automatic

CONSEQUENCE / LIMIT

- PRIVATE DOMINANCE IN CURATIVE CARE → often unregulated or under-regulated
- HIGH COSTS and unnecessary procedures

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: BALANCE → do not deny private capacity, and do not assume regulation is automatic.

08

STAGE 08

REGULATORY TOOLKIT

REGULATORY TOOLKIT | CLINICAL ESTABLISHMENTS ACT, 2010 | REGISTRATION AND REGULATION OF ESTABLISHMENTS | ITS CONDITION | STATES MUST ADOPT IT OR NOTIFY EQUIVALENT LEGISLATION; ADOPTION | PRICE CAPPING | EXERCISED OVER DEVICES BY THE PHARMACEUTICAL PRICING AUTHORITY | TREATMENT GUIDELINES

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments	PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority	RULE → name the adoption gap a regulatory recommendation is meant to close
ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies	TREATMENT GUIDELINES → promote rational care and cost control; uptake varies	CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments

SOCIAL UNIT / DEFINITION

- CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments
- ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies

DECISIVE DISTINCTION

- PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority
- TREATMENT GUIDELINES → promote rational care and cost control; uptake varies

EXAM TRAP / LIMIT

- RULE → name the adoption gap a regulatory recommendation is meant to close
- CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → name the adoption gap a regulatory recommendation is meant to close.

09

STAGE 09

FINANCING FAMILIES AND CHAIN

FINANCING FAMILIES AND CHAIN | SOCIAL HEALTH INSURANCE | CONTRIBUTION-LINKED POOLING, AS IN THE EMPLOYEES SYSTEM | GENERAL REVENUE, AS IN THE HEALTH MISSION AND THE | REVENUE COLLECTION TO RISK POOLING TO STRATEGIC PURCHASING TO | INSURANCE LEAVES PREVENTION, MEDICINES AND OUTPATIENT SPENDING UNRESOLVED | CALLING THE TAX-FUNDED PILLAR A CONTRIBUTORY INSURANCE SCHEME

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system	CHAIN → revenue collection to risk pooling to strategic purchasing to provision	TRAP → calling the tax-funded pillar a contributory insurance scheme
TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	GAP → insurance leaves prevention, medicines and outpatient spending unresolved	SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system

SOCIAL UNIT / DEFINITION

- SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system
- TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar

DECISIVE DISTINCTION

- CHAIN → revenue collection to risk pooling to strategic purchasing to provision
- GAP → insurance leaves prevention, medicines and outpatient spending unresolved

EXAM TRAP / LIMIT

- TRAP → calling the tax-funded pillar a contributory insurance scheme
- SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: TRAP → calling the tax-funded pillar a contributory insurance scheme.

10

STAGE 10

COMMUNITY HEALTH MAPPING

COMMUNITY HEALTH MAPPING | REACH THE UNDERSERVED FIRST RATHER THAN THE EASIEST TO | VILLAGE HEALTH, SANITATION AND NUTRITION COMMITTEES | INTER-SECTORAL ACTION | WATER, SANITATION, NUTRITION AND GENDER EQUALITY | APPROPRIATE TECHNOLOGY | FRONTLINE HEALTH ACTIVISTS AND AUXILIARY NURSE MIDWIVES | THE UPGRADED WELLNESS CENTRE AS THE NAMED INDIAN DELIVERY

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
EQUITY → reach the underserved first rather than the easiest to reach	INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality	INSTITUTION → the upgraded wellness centre as the named Indian delivery point
PARTICIPATION → village health, sanitation and nutrition committees	APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives	EQUITY → reach the underserved first rather than the easiest to reach

SOCIAL UNIT / DEFINITION

- EQUITY → reach the underserved first rather than the easiest to reach
- PARTICIPATION → village health, sanitation and nutrition committees

DECISIVE DISTINCTION

- INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality
- APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives

EXAM TRAP / LIMIT

- INSTITUTION → the upgraded wellness centre as the named Indian delivery point
- EQUITY → reach the underserved first rather than the easiest to reach

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: INSTITUTION → the upgraded wellness centre as the named Indian delivery point.

11

STAGE 11

CAPACITY LIMITS AND ANSWER SPINE

CAPACITY LIMITS AND ANSWER SPINE | UNION FINANCING AND STANDARDS DEPEND ON STATE DELIVERY CAPACITY | WORKFORCE VACANCIES AND UNEVEN CAPACITY LIMIT IDENTICAL NATIONAL SCHEMES | MENTAL HEALTH | A STATUTORY RIGHT AND A TELE-SERVICE STILL NEED DISTRICT | NAME THE COVERAGE DIMENSION TESTED; BUILD | PROVISION PLUS REGULATION | GRADED VERDICT WITH A DASHBOARD-COUNT CAVEAT AND A REVERSAL

CONDITION → Union financing and standards depend on State delivery capacity → **GAP → workforce vacancies and uneven capacity limit identical national schemes** → **MENTAL HEALTH → a statutory right and a tele-service still need district professionals** → **OPEN → name the coverage dimension tested; BUILD → provision plus regulation** → **CLOSE → graded verdict with a dashboard-count caveat and a reversal condition**

SOCIAL STRUCTURE / CONCEPT

- CONDITION → Union financing and standards depend on State delivery capacity
- GAP → workforce vacancies and uneven capacity limit identical national schemes

MECHANISM OF CHANGE

- MENTAL HEALTH → a statutory right and a tele-service still need district professionals
- OPEN → name the coverage dimension tested; BUILD → provision plus regulation

CONSEQUENCE / LIMIT

- CLOSE → graded verdict with a dashboard-count caveat and a reversal condition
- CONDITION → Union financing and standards depend on State delivery capacity

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: CLOSE → graded verdict with a dashboard-count caveat and a reversal condition.

E

EXTRA

SUBORDINATE ENRICHMENT — ONLY AFTER THE COMPLETE CORE

OPTIONAL ADVANCED ENRICHMENT | EXPAND HWC/AAM FOR COMPREHENSIVE PRIMARY CARE — REDUCES NEED | STRENGTHEN NHM (FREE DRUGS, DIAGNOSTICS, WORKFORCE) — IMPROVES PUBLIC | PM-JAY FOR SECONDARY/TERTIARY COVER — SHIFTS HOSPITALISATION COST FROM | REGULATORY ENFORCEMENT | CLINICAL ESTABLISHMENTS ACT, PRICE CAPS (NPPA FOR | PUBLIC-PRIVATE PARTNERSHIPS WITH ACCOUNTABILITY | EMPANEL PRIVATE PROVIDERS

OPTIONAL RIGHTS AND CAPABILITY DEPTH

- Expand HWC/AAM for comprehensive primary care — reduces need for
- Strengthen NHM (free drugs, diagnostics, workforce) — improves public
- PM-JAY for secondary/tertiary cover — shifts hospitalisation cost from
- Regulatory enforcement: Clinical Establishments Act, price caps (NPPA for

INDICATOR, ROUND AND STATUS LIMIT

- Public-private partnerships with accountability: empanel private providers
- NHP 2017: Policy, not statute; sets vision for UHC, primary-care-led
- NHM (NRHM + NUHM): Centrally-sponsored scheme; provides flexi-pool funds,
- Ayushman Bharat - HWC/AAM: Target: 1.5 lakh HWCs (upgraded SCs/PHCs);

COMPARATIVE WELFARE-DESIGN NUANCE

- Ayushman Bharat - PM-JAY: its core route uses notified eligibility criteria
- CAUTION: Clinical Establishments Act adoption: Some states have adopted or enacted
- 2024 Q1?: The question directly tests the state's role against
- PM-JAY 70+ expansion: The all-citizen 70+ route is operational, regardless of

OPTIONAL STATUS — This optional depth is unnecessary for a competent core answer; use it only for added nuance after the complete core has been written.

END OF MASTER CANVAS • poster embeds this exact master • tiled pages are overlapping crops of the same pixels